

Effective 10/1/19

I. Commercial Reimbursement

A. Inpatient Hospital Services

Hospital agrees to bill and accept as payment in full, the lesser of the rates as set out below or **Hospital's** billed charges, less any applicable co-payments and deductibles due from Member for medically necessary covered services provided to all Members.

Inpatient Per Diem/Case Rate Reimbursement

Hospital agrees that per diem/case rates shall include all related supplies, materials, radiology, laboratory, (technical only), therapies, and drug costs provided and billed by the **Hospital**

Service	Coding	Rate	Methodology
Medical / Surgical	(Rev Codes 100,101,110-113,115,117, 120-123,125,127,130-133, 135,137,140-143,145,147, 150-153,155,157,160,164, 167,190-199)	\$4,585	Per Diem
ICU/CCU/PEDS/NICU	(Rev Codes 172-174, 200-214 & 219)	\$5,725	Per Diem
OB Services		(Includes mother and baby, up to the date of the mothers discharge)	
Vaginal Delivery	DRG 768 796,797,798,805,806,807	\$8,618	Case Rate
C-Section	DRG 783,784,785,786,787,788	\$11,080	Case Rate
Boarder Baby	Revenue Codes: (Rev Codes 170, 171 & 179 - after mothers discharge)	\$1,034	Per Diem
Cardiac Case Rate	DRG 286-287	\$24,007	Case Rate
Orthopedic Case Rate	DRG 466-470	\$24,622	Case Rate
Trauma	Revenue Codes 681-684	52.81%	Percent of Charge – all inclusive

Stop Loss Provision

For Inpatient Admissions a stop loss provision shall apply. The stop loss will be \$125,214.23 in billed charges and the applicable % of charge becomes 36.56% of charges for the entire length of stay. Stop Loss will apply if calculated total charges more than the eligible_amt (column in data set).

Inpatient Carveouts

Prosthetics, Orthotics, Implants, High Cost Drugs, Revenue codes 274-276, 278 and 636. These revenue codes when billed pay at 28.72% of charges.

Service	Coding	Rate	Methodology
Implants	274-276, 278	28.72	PIA
Drugs	636	28.72	PIA

B. Outpatient Hospital Services

Hospital agrees to bill and accept as payment in full, the lesser of the rates as set out below or **Hospital's** billed charges, less any applicable co-payments and deductibles due from Member for medically necessary covered services provided to all Members.

Outpatient	Codes Required	Rate	Method
Knee Arthroscopy	CPTs: 29850-29851, 29866-29889	\$4,924	Case Rate
MRI/MRA	Revenue Codes 610, 611, 612, 619	\$1,909	Per Unit of Service
CT	Revenue Codes 350, 351, 352, 359	\$1,138	Per Unit of Service
Emergency Department ER Level I	(Rev Code 450, 451, 452, 459 and HCPC's in (099281,0G0380)	\$369	Flat Fee
Emergency Department ER Level II	(Rev Code 450, 451, 452, 459 and HCPC's in (099282,0G0381)	\$985	Flat Fee
Emergency Department ER Level III	(Rev Code 450, 451, 452, 459 and HCPC's in (099283,0G0382)	\$1,847	Flat Fee
Emergency Department ER Level IV	(Rev Code 450, 451, 452, 459 and HCPC's in (099284,0G0383)	\$3,818	Flat Fee
Emergency Department ER Level V	(Rev Code 450, 451, 452, 459 and HCPC's in (099285,099291,099292, 0G0384)	47.54%	Percent of Charge
Mammography	(Rev Code 401 or 403 and HCPCs between 0G0202-0G0206 or 077055-077059)	\$221	Per Unit of Service
Ultrasound	Revenue Code 402	\$493	Per Unit of Service

Commercial Reimbursement, continued

For all other commercial outpatient services not set out above, **Hospital** agrees to accept as payment in full 47.54% of **Hospital's** standard billed charges, less any applicable co-payments and deductibles due from Members, for Covered Services provided to all Members.

Outpatient Carveouts

Prosthetics, Orthotics, Implants, High Cost Drugs, Revenue codes 274-276, 278 and 636. These revenue codes when billed pay at 28.72% of charges.

Service	Coding	Rate	Methodology
Implants	274-276, 278	28.72	PIA
Drugs	636	28.72	PIA

C. POC/Stoploss/Carveouts Adjustments:

Offsets to Charge master (CDM) Increase

In the event that **Hospital** establishes a rate increase for its usual and customary charges, **Hospital** agrees that while this Agreement is in effect, **Hospital** shall discount these rates to **Payer** so that no higher payment shall be paid by **Payer** than it would have paid had such rate increase not been implemented. **Hospital** agrees to notify **Payer** in writing of the date and amount of any increase in its usual and customary charges no later than thirty (30) days prior to the effective date of the increase.

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